

Agent Interstitium

For the In-Between patient post ER discharge

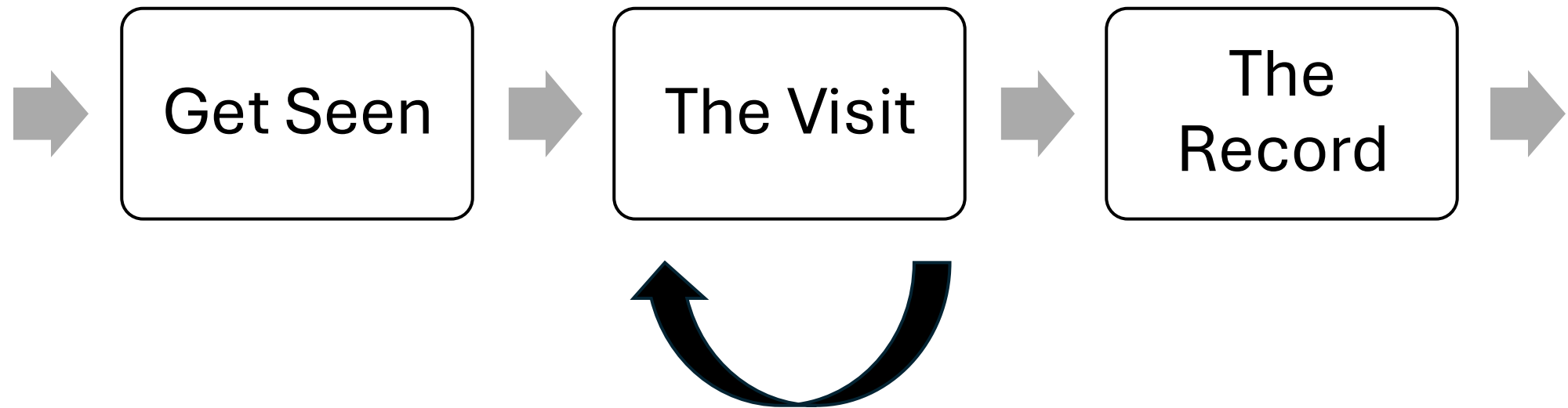
Ann-Hui Ching

ER doctor from Singapore

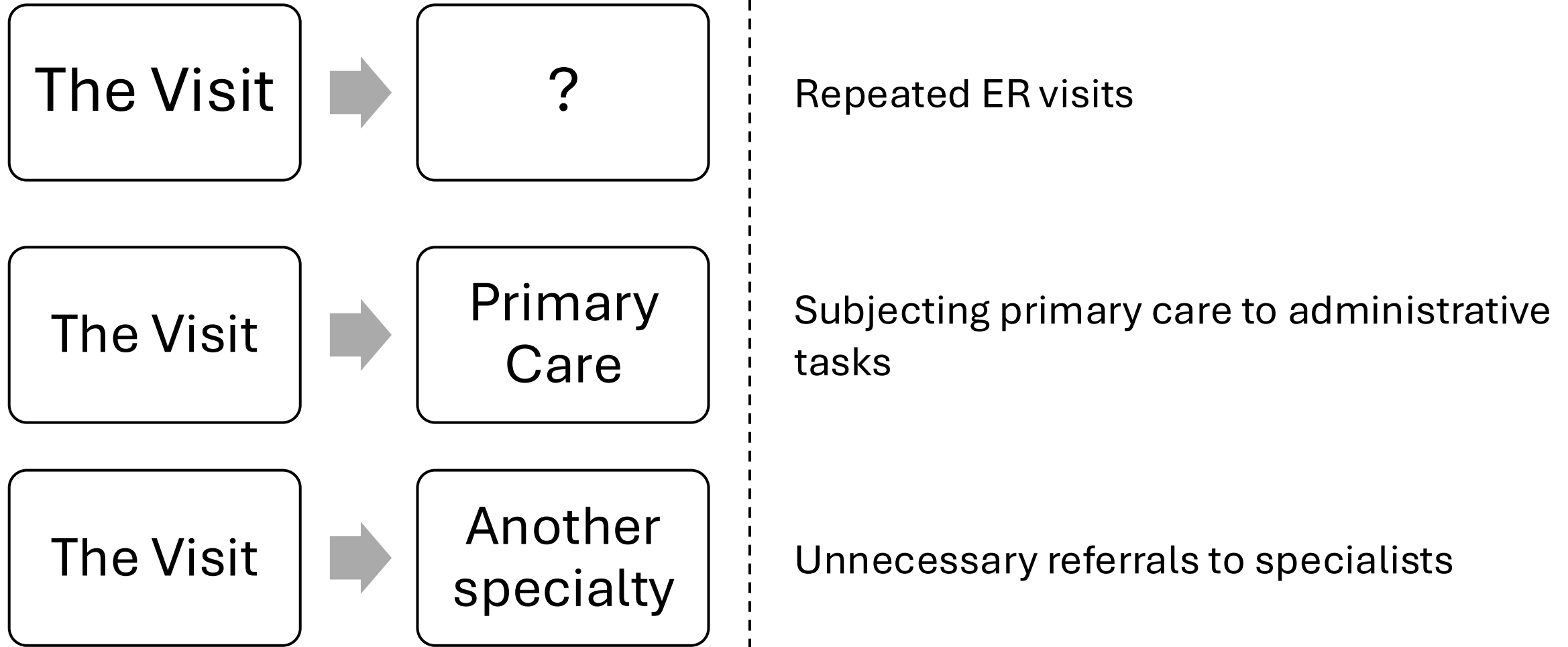
July 18 2026

Abridge Hackathon

The visit is an iterative process



Pt-Dr relationship in the ER is disrupted



Underlying assumption: The patient will show up!

The current workaround in the ER is a problem



Dedicated shifts for attending physicians and nurses to click through the records of all the patients that have been discharged!!!

Demo: Interstitium

Regulatory Challenges: Prescribing

- **Named physician's standing authorization** (they tapped "[Agent]" at discharge), escalates anything systemic to a human, and logs every action, positioning it as a *workflow tool executing a physician's plan*, not an autonomous diagnostician. The "[Both]" default is your safety story

FOLLOW-UP OWNERSHIP — ACTION REQUIRED15:11

This patient has a **pending culture** and **no PCP**. Assign follow-up owner?

Agent

Myself

Both

Safe default in production is **[Both]** — the agent works the loop, the physician stays on the thread.

Regulatory: Patient consent

- **Outbound patient contact / TCPA:** automated calls/texts need patient consent, captured at that triage verification step, which does double duty (reachability + consent).

CONTACT VERIFICATION

14:02

✓ Test SMS delivered to (415) 555-0148 at 14:02 – number confirmed reachable.

For patients experiencing homelessness, this step confirms a reachable number or a shelter/clinic callback point instead of a static address.

Regulatory: HIPAA

- **HIPAA across settings (ED, pharmacy, patient's phone, Teams):** each hop is a known quantity BAAs with every vendor, PHI encrypted at rest/transit, and critically, the **minimum-necessary principle**, which your physician ping demonstrates literally (no name/MRN/DOB in the Teams message; real PHI only behind the authenticated EHR link). Call that out as a deliberate design choice, because it is one.
 - Bonus points: Humanize the patient again, real way physicians talk to one another

The screenshot displays a Microsoft Teams chat window with a dark theme. The chat history shows three messages:

- 15:09 ESCALATION** Paging Dr. Lim via Microsoft Teams — identifiers stripped.
- 15:09 ESCALATION** A message from "Interstitium follow-up agent" at 15:09. The message body reads: "Follow-up on your ED patient from Sat 2pm — the music teacher, jazz trio, first-time UTI, sent home on cipro. Culture is **ESBL, cipro-resistant**, and she's now reporting **fever + flank pain**. Possible pyelonephritis. Please review in EHR:". Below the text is a blue button labeled "Open secure record →".
- 15:10 SYSTEM** Delivered 15:09. Read receipt 15:10.
- 15:11 ACTION** Dr. Lim tapped the secure link — authenticating into EHR.

Below the main message, a note states: "Identifiers stripped — no name, no MRN, no DOB. A memorable non-identifying social detail is included because that is how clinicians actually re-recognise a patient. The actual PHI lives only behind the authenticated EHR link."

The real value lies in the Unknowns

Known Knowns e.g. High risk AMA (AMI, Asthma exacerbations)	Known Unknowns e.g. Urine culture results
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<i>Unknown Knowns</i> e.g. Changes in radiological reports e.g. Symptoms follow-up post discharge	<i>Unknown Unknowns</i> e.g. low risk patients discharged without follow-up